



Care Indeed Home Health

Mock Patient Evaluation Document · Version 1.0 MOCK · Training Date
July 1, 2026

MOCK TRAINING RECORD
SYNTHETIC UAT DATA
NOT FOR CLINICAL USE
DO NOT UPLOAD TO LIVE PATIENT
CHART

EVALUATION 5 — EXTREME ACUITY

Primary Diagnosis: Home IV TPN via Tunneled Central Line, Short Bowel Syndrome, Enterocutaneous Fistula, Wound VAC, Chronic Pain on Methadone, History of IVDU

PATIENT DEMOGRAPHICS

Field	Detail
Patient Name	Destiny Rayne Alvarez-Jackson
Date of Birth	07/03/1988
Age	36
Sex	Female
Race/Ethnicity	Hispanic/Latina (Mexican American) and Black/African American (multiracial)
Primary Language	English
Interpreter Needed	No; however, grandmother (in-home caregiver) is Spanish-speaking only — Spanish interpreter required for all caregiver teaching
Marital Status	Single (in relationship — boyfriend not a reliable caregiver)
Medical Record Number	CI-2025-00447
Address of Service	829 Mariposa Way, Apt 6, East Palo Alto, CA 94303
Cell Phone	(650) 555-0179
County	San Mateo

EMERGENCY CONTACTS

Priority	Name	Relationship	Phone	Lives with Patient?
1	Rosa Alvarez	Grandmother (maternal) — primary caregiver	(650) 555-0180	Yes
2	Marcus Jackson	Father of patient's children	(650) 555-0544	No — but has children on weekends
3	Gloria Alvarez-Torres	Mother	(650) 555-0831	No — but visits frequently

Children in Home:

Jaylen Jackson, age 6 (kindergarten — Marcus has partial custody, with patient weekdays)

Aria Jackson, age 3 (home with patient/grandmother full-time)

Special Considerations: Two young children in home. Caregiver grandmother (age 74) is sole reliable adult support. Multiple infection risks from pediatric illnesses. Sharps and medications must be secured away from children. Wound VAC and TPN tubing — child safety concerns.

LIVING SITUATION

Field	Detail
Lives With	Grandmother Rosa (74, Spanish-only, mild HTN, arthritic but functional), 2 children (ages 6 and 3)
Residence Type	Ground-floor 2BR apartment, rented
Stairs	None
Pets	None
DME in Home	Wound VAC (KCI V.A.C. Ultra), IV pole, TPN infusion pump (Curlin 6000), tunneled Hickman catheter supplies, hospital bed (patient declined — using own queen bed), bedside commode (rarely used — patient ambulates to bathroom), digital scale, thermometer, BP monitor

PAYER INFORMATION

Priority	Payer	ID / Policy Number
Primary	Medi-Cal Managed Care (Health Plan of San Mateo)	HPSM-90412763B
Secondary	N/A	

Note: Medi-Cal managed care plan. Prior authorization required for most services. Home infusion services (TPN) authorized through Coram/CVS Specialty Infusion (contracted provider). Wound VAC rental authorized through KCI/3M.

PHYSICIAN INFORMATION

Role	Name	Specialty	Phone
Attending / Certifying	Dr. Jonathan Rivera	General Surgery / Surgical Critical Care	(650) 555-1215
GI	Dr. Heather Kim	Gastroenterology	(650) 555-1320
Pain Management	Dr. Sanjay Patel	Pain Medicine / Addiction Medicine	(650) 555-1445
Wound Care	Dr. Angela Torres	Wound Care / Hyperbaric Medicine	(650) 555-1550
Psychiatry	Dr. Nina Kovacs	Psychiatry	(650) 555-1780
Infectious Disease	Dr. Christopher Wynn	Infectious Disease	(650) 555-1655
Nutrition Support	Dr. Priya Sharma	Clinical Nutrition / TPN Management	(650) 555-1870

F2F Date: 06/12/2025 F2F By: Dr. Jonathan Rivera

REFERRAL / HOSPITALIZATION HISTORY

Event	Facility	Dates	Reason
Hospital #3	Stanford Medical Center	05/15/2025 – 06/14/2025	Exploratory laparotomy with lysis of adhesions, small bowel resection (120 cm removed — ischemic bowel secondary to mesenteric venous thrombosis); complicated by anastomotic leak → reoperation 05/22 → enterocutaneous fistula formation; short bowel syndrome (estimated 90 cm functional small bowel remaining); Hickman catheter placement for TPN; wound VAC placement
Hospital #2	Stanford Medical Center	03/08/2025 – 03/22/2025	Small bowel obstruction, adhesive (conservative management); DVT left femoral vein (provoked — immobility + OCP use); IVC filter placed
Hospital #1	San Mateo Medical Center	01/15/2025 – 01/22/2025	Small bowel obstruction, adhesive (resolved with NG decompression)
Prior Surgical History	Various	Multiple dates	Open appendectomy with perforation and peritonitis (2016); exploratory laparotomy with adhesiolysis (2019); C-section x2 (2018, 2021); cholecystectomy (2022) — extensive surgical abdomen with dense adhesions

Start of Care Date: 06/15/2025

DIAGNOSES (extremely complex — 24 active diagnoses)

Priority	ICD-10	Diagnosis
Primary	K91.2	Postsurgical malabsorption, not elsewhere classified (short bowel syndrome — 90 cm functional SB)
Surgical	K63.2	Fistula of intestine (enterocutaneous fistula — right lower quadrant, draining 400-600 mL/day)
Surgical	T81.4XXA	Infection following a procedure, not elsewhere classified (intra-abdominal abscess, drained; currently on IV antibiotics)
Active	Z93.4	Other artificial openings of gastrointestinal tract status (note: patient does not have an ostomy, but ECF functions similarly)
Active	E44.1	Mild protein-calorie malnutrition (albumin 2.4, prealbumin 12 at discharge)
Active	E86.0	Dehydration (ongoing — high-output ECF; requires IV fluid supplementation)
Active	E87.6	Hypokalemia (chronic — GI losses; requires aggressive IV and oral supplementation)
Active	E83.42	Hypomagnesemia (chronic — GI losses)
Active	E87.1	Hyponatremia (Na 132 at discharge)
Active	Z99.11	Dependence on parenteral nutrition (TPN via Hickman — 7 days/week, 14 hr/day cycling)
Active	Z45.2	Encounter for adjustment and management of vascular access device (Hickman catheter)
Active	I82.412	Acute embolism and thrombosis of left femoral vein (recent — on anticoagulation; IVC filter in place)
Active	Z79.01	Long-term (current) use of anticoagulants (enoxaparin, transitioning to warfarin)
Active	D62	Acute posthemorrhagic anemia (Hgb 8.1 at discharge; iron infusion completed inpatient)
Active	G89.29	Other chronic pain (chronic abdominal pain — adhesive disease, ECF site, surgical)
Active	F11.21	Opioid use disorder, in sustained remission (history of IV heroin use 2014-2017; methadone maintenance since 2017)
Active	Z79.891	Long-term (current) use of opiate analgesic — methadone (dual purpose: OUD maintenance + pain)
Active	F33.1	Major depressive disorder, recurrent, moderate
Active	F43.10	Post-traumatic stress disorder, unspecified (childhood trauma; medical PTSD from repeated surgeries/hospitalizations)
Active	F41.1	Generalized anxiety disorder
Active	L03.311	Cellulitis of abdominal wall (resolving — periwound; on antibiotics)
Active	N18.3	Chronic kidney disease, stage 3a (GFR 48 — AKI in hospital, partially recovered)
Active	I10	Essential hypertension
Active	K58.9	Irritable bowel syndrome (pre-existing — now complicated by short bowel)

ALLERGIES

Allergen	Reaction	Severity
Cefazolin	Anaphylaxis (intraoperative — 2016)	LIFE-THREATENING
Amoxicillin	Diffuse urticaria, throat tightness	Severe
Ciprofloxacin	QT prolongation (documented on telemetry 2025)	Severe
Ibuprofen / NSAIDs	GI bleeding (2019)	Moderate
Adhesive tape	Blistering, skin tears	Moderate
Eggs	Nausea	Mild — BUT relevant to TPN formulation (lipid emulsion) — TPN lipid source confirmed egg-free by pharmacy

CURRENT MEDICATIONS (27 medications — highest-complexity regimen)

#	Medication	Dose	Route	Frequency	Purpose	High-Risk / Critical Notes
1	TPN (Parenteral Nutrition)	Custom formula (per nutrition support RD/MD)	IV via Hickman	Continuous 14 hr/day (6 PM – 8 AM cycling)	Primary nutrition; SBS	Pharmacy mixes; delivered weekly by Coram. Contains dextrose, amino acids, lipids, electrolytes, vitamins, trace elements. Patient/grandmother trained on pump setup/disconnect. Monitor glucose during infusion (hyperglycemia) and after disconnect (hypoglycemia).
2	Lipid emulsion 20% (Intralipid)	Per TPN formula	IV (mixed in TPN or separate)	3x/week	Essential fatty acids	Egg allergy — verified egg-free lipid source (SMOFlipid)
3	Normal Saline 0.9%	1000 mL	IV via Hickman	Daily (separate from TPN — run over 4 hrs during daytime)	Volume replacement (ECF losses)	Adjust based on I&O and electrolytes
4	Potassium chloride	40 mEq	IV (in NS bag)	Daily	Hypokalemia replacement	Monitor K+ — life-threatening if hypo or hyper
5	Magnesium sulfate	2 g	IV	3x/week (M-W-F)	Hypomagnesemia	Monitor Mg levels weekly
6	Ertapenem (Invanz)	1 g	IV	Daily x 10 more days (through 06/25/2025)	Intra-abdominal infection (polymicrobial)	Cannot use cephalosporins (allergy) or ciprofloxacin (QT). Infuse over 30 min.
7	Metronidazole (Flagyl)	500 mg	IV	TID x 10 more days (through 06/25/2025)	Anaerobic coverage (intra-abdominal)	No alcohol — coordinate with addiction history. IV preferred over PO (absorption unreliable in SBS).
8	Methadone	80 mg	PO	Daily (split: 40 mg AM, 40 mg PM)	OUD maintenance + chronic pain	CONTROLLED SUBSTANCE — HIGH ALERT. Dispensed by methadone clinic (daily observed dosing transitioned to 2-week take-home during hospitalization per clinic exception). Home supply: 14-day take-home doses in lockbox. Monitor for diversion, misuse, sedation, respiratory

						depression. QTc monitoring (methadone prolongs QT).
9	Enoxaparin (Lovenox)	60 mg	SubQ	BID	DVT treatment (left femoral) + IVC filter	Transitioning to warfarin once INR therapeutic x 2 consecutive draws. Monitor anti-Xa if needed.
10	Warfarin (Coumadin)	5 mg (titrating)	PO	Daily (PM)	Anticoagulation (bridge from enoxaparin)	INR monitoring 2-3x/week during titration. Drug interaction with metronidazole (increases INR) — monitor closely. Target INR 2.0-3.0.
11	Loperamide (Imodium)	4 mg	PO	QID (before meals + HS)	Reduce GI transit time / ECF output	Max 16 mg/day; monitor for ileus
12	Cholestyramine (Questran)	4 g	PO	BID (between meals)	Bile acid malabsorption (SBS)	Give 1 hr before or 4 hrs after other oral medications — binds drugs
13	Omeprazole	40 mg	PO	BID	Gastric hypersecretion (SBS-related)	
14	Ondansetron (Zofran)	4 mg	PO/SL	Q8H PRN nausea	Nausea	QT prolongation risk — already on methadone (additive QT risk); limited use
15	Pancrelipase (Creon)	36,000 units	PO	With meals and snacks	Pancreatic enzyme supplementation (malabsorption)	Swallow whole — do not crush
16	Sertraline	100 mg	PO	Daily AM	Depression / PTSD	
17	Hydroxyzine (Vistaril)	25 mg	PO	TID PRN anxiety	Anxiety (non-benzo — chosen due to OUD history)	Sedation risk — additive with methadone
18	Trazodone	50 mg	PO	HS PRN insomnia	Insomnia	
19	Multivitamin (liquid)	15 mL	PO	Daily	Nutritional deficiency	Liquid formulation — better absorption in SBS
20	Vitamin B12	1000 mcg	SubQ	Monthly (1st of month)	B12 deficiency (terminal ileum resected)	
21	Folic acid	1 mg	PO	Daily	Folate replacement	

22	Vitamin D3	50,000 IU	PO	Weekly	Vitamin D deficiency (malabsorption)	
23	Zinc sulfate	220 mg	PO	Daily	Zinc deficiency (high-output fistula)	
24	Selenium	200 mcg	PO	Daily	Selenium deficiency (SBS)	
25	Iron sucrose (Venofer)	200 mg	IV	Weekly x 5 doses (then reassess)	Iron deficiency anemia	Infuse over 15 min.
26	Acetaminophen	650 mg	PO	Q6H PRN (max 2600 mg/day — reduced for liver health)	Pain adjunct	
27	Wound VAC supplies	Per wound care protocol	Topical/device	Continuous -125 mmHg	ECF fistula wound management	KCI V.A.C. Ultra system — dressing change 3x/week per wound care MD orders

TOTAL MEDICATIONS: 27 IV Medications: 7 (TPN, lipids, NS, KCl, MgSO₄, ertapenem, metronidazole) Controlled Substances: Methadone 80 mg daily (take-home lockbox) Sharps in Home: Yes — enoxaparin syringes, B12 syringes, insulin syringes (for TPN glucose management if added) Hickman Catheter: Dual-lumen tunneled catheter, right subclavian vein, inserted 06/05/2025. Lumen 1: TPN/lipids/NS. Lumen 2: IV antibiotics/iron/magnesium.

WOUND / FISTULA ASSESSMENT

Enterocutaneous Fistula (ECF) — Right Lower Quadrant:

Parameter	Finding
Location	RLQ, 4 cm lateral to midline, 6 cm inferior to umbilicus
Fistula Orifice	Approximately 1.5 cm diameter; visible bowel mucosa at orifice
Output	400-600 mL/day (high output — defined as >500 mL/day; fluctuates); output is bilious-green to brown liquid/semi-formed effluent
Peri wound Skin	Severely excoriated and eroded in a 5 cm radius around fistula orifice due to enzymatic damage from enteric output. Denudement, weeping, and pain (8/10 during dressing changes).
Current Management	Wound VAC (KCI V.A.C. Ultra with V.E.R.A.F.L.O. Instill & Dwell) — black foam cut to fit around fistula; fistula orifice isolated with silicone catheter draining to separate collection bag; -125 mmHg continuous; instillation therapy with NS per wound care MD orders. Dressing change 3x/week (M-W-F) by skilled nurse.
Wound VAC Alarms	Patient/grandmother educated on: low pressure alarm (leak), blockage alarm, battery alarm, canister full alarm. Written troubleshooting guide provided in English and Spanish.
Effluent Collection	Separate fistula drain bag — patient/grandmother empty and record output Q shift (AM/PM).
Nutrition Correlation	ECF output directly affects nutritional status — higher output = greater electrolyte and fluid losses = TPN adjustments needed. Weekly labs guide TPN reformulation.

Abdominal Surgical Wound:

Parameter	Finding
Location	Midline laparotomy, extending from xiphoid to suprapubic area (reopened 05/22 — initially closed; dehisced post-leak)
Total Length	28 cm
Status	Open — healing by secondary intention. Wound VAC covers both the ECF peri wound area and the open portion of the midline wound.
Wound Bed	60% granulation, 25% slough, 15% fibrinous
Depth	2.5 cm at deepest point (midline, infraumbilical)
Undermining	1.5 cm at 3 o'clock and 9 o'clock positions
Drainage (non-fistula)	Moderate serosanguineous

HOMEBOUND STATUS

CONFIRMED. Patient is severely debilitated. Connected to wound VAC 24/7 and TPN pump 14 hrs/day. Ambulates short distances only (20-30 feet) with significant fatigue and abdominal pain. Carrying wound VAC canister unit during ambulation is taxing. Leaving home requires disconnecting/managing multiple devices. Immunocompromised from malnutrition (albumin 2.4). Two young children requiring care create additional barriers to outings.

ADVANCE DIRECTIVES

Document	Status
AHCD	Yes — grandmother Rosa designated as agent (patient executed 06/2025 pre-discharge). Specific instruction: patient wants full aggressive treatment including ventilator; does NOT want prolonged CPR >30 minutes if no response.
POLST	Yes — Full treatment.

SERVICES ORDERED

Service	Discipline	Frequency	Duration
Skilled Nursing	RN	Daily x 4 weeks (IV meds, wound VAC, Hickman care, labs, TPN monitoring, teaching), then 3x/week ongoing	Indefinite (minimum 60-day cert; expected ongoing need)
Physical Therapy	PT	3x/week x 8 weeks	8 weeks
Occupational Therapy	OT	2x/week x 4 weeks	4 weeks
Medical Social Work	MSW	1x/week x 8 weeks	8 weeks
Home Health Aide	HHA	Daily (personal care, child safety monitoring, light housekeeping, meal support)	Duration of need
Registered Dietitian (if ordered/available)	RD	1x/week x 4 weeks, then biweekly	8 weeks

PLAN OF CARE GOALS (60-DAY CERTIFICATION PERIOD)

#	Goal	Target
1	ECF output reduced to <200 mL/day (fistula maturation/reduction)	08/14/2025
2	Abdominal wound reduced in size by ≥30% (measurements)	08/14/2025
3	Periwound skin excoriation improved (intact periwound skin ≥50% circumference)	08/14/2025
4	No CLABSI or Hickman catheter complications	08/14/2025
5	Electrolytes stable (K+ 3.5-5.0, Mg 1.5-2.5, Na 135-145) for ≥70% of draws	08/14/2025
6	Albumin improved to ≥2.8 (from 2.4)	08/14/2025
7	Patient weight stable or gaining (target ≥125 lbs — currently 118)	08/14/2025
8	Intra-abdominal infection resolved (afebrile, WBC normal, antibiotics completed)	06/25/2025
9	Patient/grandmother demonstrate TPN pump setup, connect, disconnect, and Hickman care independently	07/15/2025
10	Patient/grandmother demonstrate wound VAC troubleshooting	07/15/2025
11	Anticoagulation stable (INR 2.0-3.0 on warfarin; enoxaparin discontinued)	07/15/2025
12	Zero substance relapse; methadone compliance verified	08/14/2025
13	Ambulate 150 feet without significant pain or fatigue	08/14/2025
14	PHQ-9 decreased to ≤10 (from current estimated 16)	08/14/2025
15	Child safety plan in place (sharps/meds/equipment secured)	06/22/2025
16	Zero falls, zero ED visits, zero readmissions	08/14/2025

PATIENT SIGNATURES — EVALUATION 5

Patient Signature: _____ Date: _____

Print Name: Destiny Rayne Alvarez-Jackson

Caregiver / Agent Signature: _____ Date: _____

Print Name: Rosa Alvarez

Interpreter Used: ☐ Yes — Spanish telephone interpreter (for grandmother teaching)

Clinician Signature: _____ Date: _____

Patient / Representative / Conservator

Mock signature area — training only.

Evaluating Clinician

Mock signature area — training only.

Caregiver / Interpreter if applicable

Mock signature area — training only.

Date / Attestation

Mock signature area — training only.